Clinical Escalation and Scope-of-Practice Standard

OPERATIONS POLICY: MHP-OP-204
SUBJECT: Scope of Practice, Clinical Communication Boundaries, and Inquiry Routing for Non-Clinical Personnel and Automated Systems
EFFECTIVE DATE: October 1, 2024
ISSUED BY: Dr. Aris Thorne, Chief Medical Officer, and Brenda Vance, Vice President of Clinic Operations, Meridian Health Partners

1. PURPOSE AND APPLICABILITY

1.1 This policy defines the strict operational boundaries governing communications between patients and all non-clinical staff (including receptionists, schedulers, billing personnel, administrative assistants, and call center representatives) as well as automated patient-facing systems (including interactive voice response systems, web chat widgets, mobile application chat interfaces, and automated email reply bots) across all 14 Meridian Health Partners (MHP) clinic sites.

1.2 This policy applies universally to all interactions conducted in person, via telephone, through the MHP Patient Portal, via SMS or secure messaging, and through automated digital interfaces.

2. DEFINITIONS

2.1 Non-Clinical Staff: Any MHP employee or contractor who does not hold a current, active license as a Physician (MD/DO), Physician Assistant (PA), Nurse Practitioner (NP), Registered Nurse (RN), Licensed Practical Nurse (LPN), or Licensed Vocational Nurse (LVN).

2.2 Automated System: Any software, algorithm, artificial intelligence model, or pre-programmed digital workflow deployed by MHP to interact with patients without direct real-time human mediation by a licensed clinician.

2.3 Licensed Clinician: An MHP-credentialed provider possessing an active state medical or nursing license authorized to provide direct patient care within their designated scope of practice.

3. PROHIBITED COMMUNICATIONS FOR NON-CLINICAL STAFF AND AUTOMATED SYSTEMS

Non-clinical staff and automated systems are strictly prohibited from providing medical advice, clinical interpretations, or treatment recommendations. Specifically, they must not:

* 3.1 Confirm, deny, or speculate on the clinical significance of any symptom, vital sign, or physical finding reported by a patient.
* 3.2 Advise patients on the therapeutic efficacy, side effect profiles, interactions, or indications of any prescription or over-the-counter medication.
* 3.3 Instruct a patient to alter, increase, decrease, or cease the dosage or administration schedule of any prescribed medication.
* 3.4 Explain, interpret, or contextualize the clinical meaning of laboratory test results, imaging reports, pathology findings, or diagnostic summaries, even if values are flagged as "normal" or "within reference range" in the electronic health record (EHR).
* 3.5 Answer, evaluate, or provide guidance on any inquiry phrased by the patient with the words "should I" (e.g., "Should I go to the ER?", "Should I stop taking this pill?", "Should I wait to see if this gets better?").

4. MANDATORY ROUTING CATEGORIES

Any patient inquiry falling into one of the following five categories must be immediately intercepted by automated systems or transferred by non-clinical staff to a licensed clinician or clinical triage queue within the timelines specified in Section 6:

* Category A: Symptom interpretation or triage.
* Category B: Medication questions (efficacy, indications, side effects).
* Category C: Dosage instructions or adjustments.
* Category D: Test-result meaning and diagnostic interpretation.
* Category E: Any question phrased as "should I" regarding health status, treatment, or care settings.

5. MANDATORY DEFLECTION SCRIPT

5.1 When a patient presents an inquiry falling under Section 3 or Section 4, non-clinical staff and automated systems must immediately cease providing further information regarding the subject matter and deploy the exact, unedited deflection script set forth below.

5.2 Required Deflection Script (Verbatim):

> *"I am not a licensed clinician, and under Meridian Health Partners policy, I am not permitted to interpret symptoms, discuss medications, explain test results, or advise you on care decisions. Let me connect you directly with a clinical triage nurse or route your question to your care team so a qualified professional can assist you."*

5.3 No paraphrasing, summarizing, or truncation of the deflection script is permitted. Automated systems must output this exact string as a text block or audio prompt before initiating the transfer or routing ticket.

6. ROUTING TIMELINES AND PROTOCOLS

6.1 For telephone interactions, non-clinical staff must warm-transfer the caller to the clinical triage queue for the patient’s home clinic site within 90 seconds. If the clinical queue is unavailable, staff must generate an urgent clinical encounter ticket in the EHR with a 15-minute callback SLA.

6.2 For digital, portal, or automated system interactions, the system must automatically generate a clinical routing task assigned to the primary care provider's nurse pool within 120 seconds of the prohibited query being detected.

7. ZERO-EXCEPTION RULE

7.1 This policy operates under a strict zero-exception mandate.

7.2 The prohibitions in Section 3 and the mandatory routing and scripting requirements in Sections 5 and 6 apply without exception under all circumstances, including but not limited to:

* 7.2.1 Repeat callers who have asked the same question previously and received a clinical answer.
* 7.2.2 Patients who explicitly state, demonstrate, or document that they are themselves licensed healthcare professionals (e.g., physicians, nurses, pharmacists, paramedics).
* 7.2.3 Emergency or urgent scenarios where immediate self-reporting is required (in suspected life-threatening emergencies, staff must bypass routing and immediately instruct the patient to dial 911, while still adhering to the non-clinical scope boundary).
* 7.2.4 Urgent care follow-ups or administrative inquiries that touch upon clinical parameters.

8. COMPLIANCE AND ENFORCEMENT

8.1 All inbound and outbound call recordings, portal messages, and automated chat logs are subject to random monthly audit by the MHP Quality and Compliance Directorate.

8.2 Failure by any non-clinical employee to adhere to the deflection script or unauthorized provision of clinical guidance will result in progressive disciplinary action up to and including immediate termination of employment, in accordance with MHP Human Resources Policy HR-802.

8.3 Automated systems found to be generating prohibited clinical interpretations shall be immediately quarantined and disabled by the MHP Health Informatics Department until algorithmic re-validation is completed by the Clinical Operations Committee.